Carbon Monoxide / Cyanide

TE-02 | Toxicology & Environmental

Quick flow

01 Scene Safety + Decontaminate [EMT / AEMT / Paramedic]

Stage or request PPE/HAZMAT support, remove the patient from exposure, and decontaminate liquid or industrial cyanide contamination before ambulance transport when indicated.

02 100% Oxygen [EMT / AEMT / Paramedic]

Give regardless of standard pulse-ox reading; support airway and ventilation.

03 Assess Toxicity [EMT / AEMT / Paramedic]

Assess for altered mental status, dyspnea, weakness, nausea/vomiting, syncope, seizure, chest pain, shock, neurologic deficit, or respiratory failure.

04 Monitor + Access [AEMT / Paramedic]

Obtain glucose, 12-lead, IV/IO, and cardiac monitoring as indicated. Use pulse CO-oximetry when available, but do not delay care or rely on one field SpCO value to rule out poisoning.

05 High Cyanide Suspicion? [EMT / AEMT / Paramedic]

Enclosed-space fire, known industrial cyanide exposure, or ingestion with severe AMS, seizure, hypotension/collapse, or respiratory failure.

06 Consult + Early Alert [EMT / AEMT / Paramedic]

Call Poison Help (1-800-222-1222) and Medical Control for severe exposure; notify the receiving facility early. Claiborne ground units do not stock a cyanide antidote.

Overview

- Consider carbon monoxide and cyanide in any combustion exposure; conventional pulse oximetry and environmental readings do not exclude poisoning.
- Immediate removal from exposure, decontamination when indicated, and 100% oxygen are foundational.

Provider-Level Actions

Protect rescuers, stop exposure when safe, support ABCs, and involve Poison Control or Medical Control early.

EMT

- Remove from exposure, provide 100% oxygen and ventilation support, obtain glucose, and rapidly transport.

AEMT

- Perform all EMT care plus IV/IO access, 12-lead acquisition, and treatment of glucose abnormality or shock.

Paramedic

- Perform all prior care plus cardiac and pulse CO-oximetry monitoring when available, Poison Help/Medical Control consultation, and early destination notification for severe exposure.

Indications

- Smoke/combustion exposure, industrial cyanide exposure, cyanide ingestion, or compatible unexplained illness.

Warnings / precautions

- Provide 100% oxygen regardless of conventional pulse-oximetry reading; standard SpO_2 is unreliable in carbon monoxide exposure.

- Do not place a patient with liquid or industrial cyanide contamination in the ambulance before indicated decontamination and HAZMAT direction.
- For severe carbon monoxide exposure - loss of consciousness, neurologic deficit, dysrhythmia/ischemia, severe acidosis, or pregnancy - notify the receiving facility early for hyperbaric-consultation consideration.
- A cyanide antidote is not stocked on Claiborne ground units. Do not delay supportive care, Poison Help/Medical Control consultation, or transport while seeking an unavailable antidote.
- Pregnant patients, children, and older adults may become symptomatic at lower CO levels.

Source and references

Claiborne County EMS approved protocol manual. | Poison Help national line: 1-800-222-1222. | Current Tennessee EMS scope of practice and Claiborne County standing orders control for all provider permissions.

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